

PLANNING A PROGRAM EVALUATION

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One of the great mistakes is to judge policies and programs by their intentions rather than their results.
—Milton Friedman

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INTRODUCTION

Why Program Evaluation Matters

Most healthcare programs are well intended and are designed in what appears plausible and reasonable to address a problem. Unfortunately, good intentions and intuitive plausible interventions do not necessarily lead to better outcomes (Rossi et al., 2019). Therefore, not only is evaluation a critical aspect of program development and implementation, but it should also be conducted in a systematic science-based manner to support the reliability and validity of the results. Evidence-based practices (EBPs) and best practices are critical to sound evaluation.

This chapter discusses both program and project evaluation. Although project and program evaluations all follow basic principles and models for evaluation, *project evaluations* require much less detail and follow a simpler evaluation design, while the term *program evaluation* is used most frequently to refer to both program and project evaluations. In this chapter, both terms are used in ways that seem appropriate to the discussion. This chapter presents the core concepts of comprehensive evaluation through the lens of a Doctor of Nursing Practice (DNP) student project, using the project experience as a practical framework for understanding evaluation in real-world settings. The knowledge and skills outlined here represent essential competencies that DNP graduates are expected to apply in practice as they assess programs, guide improvement efforts, and influence system-level outcomes.

The Role of the Doctor of Nursing Practice in Program Evaluation

DNP-prepared nurses play a key role in leading and contributing to comprehensive program evaluation in healthcare organizations. As clinical and systems-level leaders, DNPs evaluate a wide range of initiatives, from focused clinical

programs, such as heart failure management services, to complex, multisite efforts within large health systems, including population health and integrated care programs.

Program evaluation expertise enables DNP-prepared nurses to assess implementation, determine whether programs achieve intended outcomes, and examine effects on quality, safety, equity, and resource use. DNPs use evaluation findings to inform organizational decision-making, guide improvement efforts, and support evidence-informed changes that advance high-quality, cost-effective care.

The American Association of Colleges of Nursing (AACN) *Essentials* (AACN, 2021) identify program evaluation as a core competency for DNP graduates, emphasizing the use of evidence-based frameworks, analytic methods, quality improvement (QI) tools, and systems thinking. These competencies prepare DNPs to conduct rigorous evaluations and translate findings into actionable recommendations that support sustainable system improvement.

Table 2.1 highlights the alignment between the DNP role and AACN Essentials Domains and Level 2 sub-competencies that support evaluation of healthcare programs, practice changes, and DNP projects.

The Doctor of Nursing Practice Project and Program Evaluation

The DNP project provides a structured, practice-immersive opportunity for students to develop and apply program evaluation competencies essential to the role of the DNP. Whether the project involves a comprehensive evaluation of a healthcare system or the implementation of a QI initiative, students engage in scholarly inquiry, evidence appraisal, data analysis, and outcome assessment that are skills foundational to rigorous evaluation (Rossi et al., 2019). This work strengthens the DNP students' ability to determine whether interventions are effective, efficient, and aligned with organizational needs.

TABLE 2.1 Alignment of AACN Essentials Domains With Program Evaluation Competencies

AACN Essentials Domain (2021)	Relevant Level 2 Sub-Competencies	Why Evaluation Matters
Domain 4: Scholarship for the Nursing Discipline	4.1b: Applies evidence-based frameworks to evaluate practice and outcomes. 4.2b: Uses analytic methods to appraise evidence and guide decision-making. 4.3b: Synthesizes data to inform practice improvements.	Ensures evaluation activities are grounded in rigorous evidence and scholarly methods. Strengthens the DNP's ability to judge intervention effectiveness and translate findings into practice. Supports development of high-quality, evidence-informed recommendations for system improvement.
Domain 5: Quality and Safety	5.1b: Uses QI methods to evaluate care processes and outcomes. 5.2b: Applies safety science to assess risks and system vulnerabilities. 5.3b: Uses data to monitor outcomes and identify improvement opportunities.	Provides structured methods for evaluating whether programs and projects improve quality and safety. Helps DNPs identify gaps, unintended consequences, and system failures. Ensures evaluation findings directly inform safer, more reliable care delivery.
Domain 7: Systems-Based Practice	Analyzes how organizational systems influence outcomes and resource use. Evaluates the impact of policies, regulations, and reimbursement on practice. Applies systems thinking to improve efficiency and sustainability.	Enables evaluation of programs within the broader organizational and policy context. Supports assessment of financial, operational, and structural factors affecting outcomes. Helps DNPs design evaluations that consider system-level interactions and sustainability.
Domain 10: Personal, Professional, and Leadership Development	Demonstrates leadership in organizational and financial decision-making. Applies change-management strategies to lead improvements. Engages in strategic planning and evaluation of outcomes.	Builds leadership capacity to design, implement, and evaluate change initiatives. Ensures DNPs can interpret evaluation results to guide strategic decisions. Strengthens the profession's ability to lead sustainable, outcome-driven improvements.

AACN, American Association of Colleges of Nursing; DNP, Doctor of Nursing Practice; QI, quality improvement.

Through a DNP project, the student learns to apply evaluation frameworks, select appropriate indicators such as measurable outcomes, process measures or patient-level results, and interpret evaluation findings in real clinical settings. The DNP project experience deepens the DNP students' understanding of how institutional context, stakeholder engagement, and system dynamics influence an evaluation, affect clinical outcomes, preparing them to lead evidence-informed improvements across complex healthcare environments (AACN, 2021; Fain, 2020). As a result, graduates enter the profession equipped to design, implement, and evaluate initiatives that advance quality, safety, and population health.

FOUNDATIONS OF PROGRAM EVALUATION

Program Evaluation Defined

Program evaluation is the systematic application of evaluation theories and principles to assess the design, implementation, and outcomes of programs or projects. Programs may range from small, single-site activities to complex, multi-site initiatives spanning multiple organizational levels and sectors (Wholey et al., 2015; W.K. Kellogg Foundation, 2004). Evaluation methods provide the tools and approaches needed to understand how a program

functions and the results it produces, whether through ongoing monitoring—such as tracking participation or service delivery—or focused assessments that examine implementation processes, short- and long-term outcomes, or broader population-level impacts.

The purpose of program evaluation is to generate credible, actionable information that supports program development, improvement, and evidence-informed decision-making. By collecting valid and reliable data using established methodologies and professional standards, evaluators can answer key questions about program performance, clarify how well the program is functioning, and identify areas for improvement (Wholey et al., 2015; W.K. Kellogg Foundation, 2017b). Effective evaluation also requires understanding the program's intended outcomes and the logic linking activities to results, ensuring that judgments about program success or gaps are evidence-based (Weiss, 1998). Ultimately, program evaluation informs decisions about maintaining, modifying, or discontinuing a program, guides resource allocation, and strengthens overall program effectiveness.

Outcomes of a Program Evaluation

The primary outcome of a program evaluation is the generation of clear, credible evidence about how well a

program is functioning and the degree to which the program achieves its desired goals and outcomes. Program evaluation provides stakeholders with the information needed to make informed decisions about program direction and resource allocation. The evaluation process examines a program's outcomes and identifies where improvements may be needed. Using systematic qualitative and quantitative methods grounded in social science research, evaluators gather valid and reliable data that address key questions about program performance (Wholey et al., 2015).

Evaluation is inherently judgment-oriented as the process ends in conclusions about the merit or value of a program. Unlike traditional research, which is typically driven by investigator-defined questions, evaluation is designed to meet the information needs of specific stakeholders. Because of this user-focused purpose, evaluators collaborate closely with program leaders and staff to ensure that the evaluation plan reflects the program's context, priorities, and decision-making needs. The overarching goal of evaluation is to support program development, implementation, and improvement by generating evidence that can guide action. As Stufflebeam and colleagues (1971) emphasized, the purpose of evaluation is “not to prove but to improve.”

Exhibit 2.1 highlights the typical results that a high-quality program evaluation should produce (Stufflebeam et al., 1971; Wholey et al., 2015; W.K. Kellogg Foundation, 2004).

Exhibit 2.1 Outcomes of a Program Evaluation
A high-quality program evaluation produces: Clear evidence of program effectiveness. Identifies whether the program is achieving its intended goals and outcomes. Insight into program strengths and weaknesses. Highlights which components are working well and which require improvement. Understanding of how program components contribute to results. Clarifies the relative impact of different activities or strategies (Wholey et al., 2015). Information to support decision-making. Helps stakeholders determine whether to continue, modify, expand, or discontinue a program (W.K. Kellogg Foundation, 2017a). Judgments based on established criteria. Uses systematic methods to assess program processes and outcomes with rigor comparable to research. Credible, actionable recommendations. Provides guidance for strengthening processes, addressing deficiencies, and improving overall performance. A foundation for future improvement Reinforces Stufflebeam's principle that the purpose of evaluation is “not to prove but to improve” (Stufflebeam et al., 1971).

Program Evaluation Versus Program Assessment

A distinction can be made between program evaluation and program assessment, although the two share some methodological similarities. Program evaluation examines the overall merit, worth, and effectiveness of a program by analyzing its structure, implementation processes, and outcomes in relation to established criteria or intended goals (Fitzpatrick et al., 2023; Wholey et al., 2015). Evaluation is decision-oriented and focuses on how well a program functions as a system, emphasizing improvement, accountability, and the production of actionable findings for stakeholders.

In contrast, program assessment typically focuses on the performance of specific components within a program—often individuals, teams, or discrete activities. Assessment measures proficiency, skill levels, or compliance with defined standards or guidelines and is commonly used to determine whether participants meet expected competencies or performance benchmarks (Suskie, 2018). While an evaluation looks at the whole program, an assessment concentrates on the performance parts of a program. Despite these differences, both evaluation and assessment rely on systematic data collection, use evidence to inform judgments, and contribute to program improvement. Assessment findings may feed into a broader evaluation, and evaluation results may highlight areas where further assessment is needed. Together, they provide complementary perspectives on program quality and effectiveness.

Types of Program Evaluations

In the context of healthcare program assessment, a “*type of evaluation*” refers to the specific approach chosen to examine particular aspects of a program, such as its design, implementation, outcomes, or overall impact. Selecting the appropriate type of evaluation is a critical first step because different evaluation types examine different aspects of a program and address distinct questions about its performance and effectiveness (Fitzpatrick et al., 2011; Hickey, 2022; Newcomer et al., 2015).

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The choice of evaluation type should align with the program's stage and the objectives for evaluating the program. For example, programs in their early-stage of implementation may benefit from formative evaluation to identify implementation challenges, whereas an outcome or summative evaluation would be used to evaluate a mature program to assess the effectiveness of the staff and processes that are in place and the degree to which the program is meeting its desired goals and objectives (Martiniano & Harasta, 2017; USEPA, 2025; W.K. Kellogg Foundation, 2017a).

The following evaluation types are frequently used in healthcare programs, with each type addressing different aspects of program performance:

- Descriptive evaluation provides qualitative and quantitative information to describe a program, without assessing effectiveness or causality.

- Process evaluation examines how a program is implemented, identifies barriers and facilitators, and supports continuous improvement.
- Formative evaluation is conducted during early stages or mid-implementation to assess feasibility, workflow, resources, and staff readiness.
- Outcome evaluation measures whether a program achieves its intended short-term or intermediate results and links changes to program activities.
- Impact evaluation is a subset of outcome evaluation that investigates causal relationships between program activities and outcomes.
- Summative evaluation assesses overall results, impact, and effectiveness at or near program completion.
- Cost-effectiveness evaluation examines whether the program's outcomes justify the resources and costs expended, often comparing alternatives.
- Needs assessment/gap analysis identifies the difference between current conditions and desired outcomes, highlighting areas for improvement or program development (Hickey, 2022; Martiniano & Harasta, 2017; USEPA, 2025).

Methodology in Program Evaluation

In program evaluation, *methodology* refers to the set of methods, procedures, and techniques used to collect and analyze data to answer specific evaluation questions. While the type of evaluation determines *what* aspect of a program is being assessed such as process, outcomes, or impact, methodology specifies *how* the evaluation is conducted, including the design, data collection methods, and analytic strategies (W.K. Kellogg Foundation, 2017a).

After identifying the appropriate type of evaluation, evaluators select methodologies that align with the evaluation's objectives, available resources, and the complexity of the program. For example, a formative evaluation of a new healthcare intervention might use case studies, interviews, or workflow observations to assess feasibility and guide improvements, whereas an outcome evaluation of a mature program might use quasi-experimental designs, cross-case comparisons, or outcome mapping to assess effectiveness and impact (W.K. Kellogg Foundation, 2004, 2017a).

By clearly distinguishing evaluation type from methodology, evaluators can ensure that the chosen methods effectively address the evaluation questions and produce evidence that informs actionable decisions, policy revisions, and program improvements. [Table 2.2](#) summarizes common program evaluation types and illustrates how each type differs in focus, example measures, and methodological approaches.

Components of a Program Evaluation

Evaluation-focused DNP projects include many of the same elements or components as QI/EBP initiatives such as partner engagement, context analysis, problem definition, literature synthesis, and the development of process and outcome objectives. However, an evaluation project may require different project components such as evaluation questions, selecting an evaluation framework rather than an implementation framework, and an implementation phase centered on executing the evaluation plan itself (Howard-Ruben et al., 2025). [Table 2.3](#) contrasts the major elements of DNP projects that center on evaluating an existing program with those that emphasize implementing a new intervention, highlighting differences in purpose, design, data needs, and expected outcomes (Hickey, 2022; Howard-Ruben et al., 2025).

TABLE 2.2 Common Program Evaluation Types, Focus Areas, and Methodological Approaches

Evaluation Type	Focus / Purpose	Examples of Measures	Methodology Examples
Formative	Feasibility, implementation, improvement	Workflow efficiency, staff readiness	Observations, interviews, process mapping
Summative	Overall outcomes and impact	Patient outcomes, program goals	Pre-/post-analysis, quasi-experimental studies
Outcome / Impact	Program effectiveness, causal relationships	Clinical metrics, population health indicators	Regression analysis, longitudinal studies
Process	Implementation fidelity	Adherence rates, service delivery	Audits, focus groups
Descriptive	Program description	Program reach, structure	Surveys, document review
Needs assessment / gap analysis	Identify gaps in services or resources	Unmet needs, disparities	Surveys, community assessments
Cost-effectiveness	Resource allocation and value	Cost per outcome, ROI	Economic modeling, comparative analysis

ROI, return on investment.

TABLE 2.3 Comparison of Components: Evaluation-Focused Projects Versus Implementation-Focused DNP Projects

Component	Evaluation-Focused DNP Project	Implementation-Focused DNP Project
Purpose and scope	Assess an existing program/process for effectiveness, gaps, and improvement opportunities.	Implement a new intervention or practice change and measure its impact.
Leader engagement	Early involvement to clarify evaluation priorities and ensure utility of findings.	Early involvement to secure buy-in for change and facilitate adoption.
Contextual analysis	Review current operations, resources, and readiness for evaluation.	Analyze organizational readiness for change and barriers to implementation.
Problem statement	Justifies need for evaluation based on performance questions or sustainability concerns.	Justifies need for intervention based on identified clinical or operational gaps.
Evaluation questions	Develop overarching and specific questions aligned with framework and stakeholder priorities.	Define objectives for implementation and improvement; questions focus on fidelity and outcomes.
Framework selection	Select evaluation model (e.g., CDC, Donabedian, Kellogg) to guide design and interpretation.	Select QI or EBP framework (e.g., PDSA, Iowa Model) to guide implementation strategy.
Design and methodology	Emphasizes formative and summative evaluation; mixed methods are often used.	Focuses on intervention design and process monitoring may include outcome measures post-implementation.
Data plan	Identify sources for process and outcome evaluation; plan for acquisition, management, and analysis.	Identify baseline and post-intervention data sources; plan for monitoring fidelity and outcomes.
Implementation activities	Execute evaluation plan (audits, surveys, interviews); monitor fidelity of evaluation processes.	Execute intervention activities (training, workflow changes); monitor fidelity of implementation.
Reporting and dissemination	Prepare written report (title, executive summary, purpose, design, results, recommendations) and oral presentation tailored to audience.	Prepare report summarizing implementation processes, outcomes, and lessons learned; share with stakeholders.
Recommendations	Provide actionable steps for improvement based on evaluation findings; include timelines and resource needs.	Recommend sustaining, scaling, or modifying the implemented intervention based on results.

CDC, Centers for Disease Control and Prevention; DNP, Doctor of Nursing Practice; EBP, evidence-based practice; QI, quality improvement; PDSA, Plan-Do-Study-Act.

Source: Howard-Ruben, J., Hinch, B., & Livesay, S. (2025). The value of comprehensive evaluation DNP projects. *Journal of Professional Nursing*, 57, 133–138. <https://doi.org/10.1016/j.profnurs.2025.01.007>; Hickey, J. V. (2022). Planning a program or project evaluation. In J. V. Hickey & E. R. Giardino (Eds.), *Evaluation of health care quality for DNPs* (3rd ed.). Springer Publishing.

Institutional Review Board Review

DNP projects that are comprehensive evaluations of a system or a program are subject to a review of the project by the organization's Institutional Review Board (IRB). IRBs vary widely in how they oversee DNP projects that involve program evaluation. Because program evaluations are typically designed to assess existing services rather than generate generalizable knowledge, IRB review often differs from that required for traditional research studies and determines that program evaluation activities do not constitute human subjects research and therefore do not require full oversight of project implementation. However, evaluators should not assume full exemption from review of all project components. IRBs usually require submission of project materials such as surveys, interview guides, recruitment scripts, or data collection procedures to ensure

that participant rights, privacy, and confidentiality are protected. When human participants are involved, even minimally, IRB review provides essential ethical safeguards and helps confirm that data collection methods pose minimal risk and are aligned with organizational policies (White et al., 2021).

PROJECT MANAGEMENT

Effective project management is essential for a successful program evaluation. It involves planning, coordinating, and monitoring all evaluation activities to ensure alignment with the project's purpose and objectives (Hickey, 2022; Hickey & Giardino, 2023). In DNP projects, the student often serves as the evaluator-manager, overseeing personnel, resources, timelines, and data management

while maintaining project momentum. Visual tools, such as Gantt charts, help track progress, anticipate delays, and guide adjustments. Frequent interim assessments identify emerging barriers, maintain scope, and ensure fidelity to the evaluation plan. Clear, consistent, and transparent communication with stakeholders throughout planning, implementation, and dissemination is critical to success.

Key responsibilities include:

- organizing and sequencing evaluation activities logically
- assigning responsibilities to clarify roles and maintain accountability
- establishing realistic timelines and monitoring progress
- using visual tools to track tasks and adjust timelines as needed
- monitoring workflow and barriers, making timely adjustments
- maintaining consistent, transparent communication with stakeholders
- ensuring all activities align with the evaluation purpose
- leading the project from start to finish as the evaluator-manager

Development Phase of a Program Evaluation

The development phase of a comprehensive program evaluation involves developing the foundational components that guide the direction, structure, and rigor of the evaluation. This phase clarifies why the evaluation is being conducted, who needs to be involved, and what contextual factors must be understood before data collection begins. Key project components such as defining the purpose and scope, engaging leaders, framework selection, and assessing environmental influences work together to ensure the evaluation is grounded in evidence, aligned with organizational priorities, and positioned for meaningful and actionable findings.

Assessment of Evaluability

An evaluation project frequently begins with an evaluability assessment. It is a critical preliminary step to clarify if stakeholders feel there is a need for an evaluation and a desire of stakeholders to use the evaluation findings (Howard-Ruben et al. et al., 2025). Wholey et al. (2015) identify four essential standards that ensure a program is evaluable and that the resulting findings will be meaningful and actionable: (a) consensus on realistic program goals, (b) clearly articulated information needs, (c) availability of relevant and reliable data, and (d) collaborator commitment to utilizing the evaluation results. Conducting an

evaluability assessment provides a structured approach to determine whether these conditions are met prior to initiating a full evaluation (Howard-Ruben et al., 2025).

For DNP students, completing an evaluability assessment provides a structured opportunity to develop essential DNP evaluation competencies. The process helps students understand how leaders view the program's purpose, implementation strategies, outputs, and intended outcomes, offering insight into the assumptions and expectations that shape program performance. It also requires students to critically appraise program operations and determine whether the program is ready for a full evaluation, strengthening their skills in stakeholder engagement, systems analysis, and evaluative thinking. Completing an evaluability assessment helps DNP students develop the skills for conducting rigorous evaluations within complex healthcare environments.

Frameworks in Evaluation Projects

Frameworks provide structure and guidance for program evaluation, QI, and EBP initiatives. They help evaluators break a complex program evaluation into manageable components, clarify relationships between key elements, and make the assumptions, values, and priorities underlying the work explicit (Frye & Hemmer, 2012; Partelow, 2023). By offering a clear conceptual map, frameworks support systematic decision-making, maintain focus on evaluation objectives, and ensure alignment with evidence and theory (James Bell Associates, 2018).

Using a framework allows evaluators to:

- organize program components and processes logically;
- illustrate expected pathways of change or outcomes;
- select appropriate measures and data collection strategies;
- incorporate contextual factors such as organizational culture, resources, and population needs; and
- facilitate communication and shared understanding among stakeholders.

In practice, frameworks enhance the clarity, feasibility, and rigor of an evaluation by making the relationships between activities, outputs, and intended outcomes explicit. Engaging stakeholders in applying a framework further strengthens evaluations by incorporating practical insights and contextual considerations, increasing the likelihood of actionable and meaningful results (McCreight et al., 2019; Ogunyemi et al., 2025).

Chapter 3, “Models and Frameworks for Evaluation in DNP Projects,” offers detailed explanations of models and frameworks commonly utilized in evaluation and implementation projects.

Table 2.4 outlines common evaluation offering a concise comparison to help DNP students determine which approach best fits the needs and context of their project.

TABLE 2.4 Common Evaluation Frameworks

Framework	Considerations
CIPP Model	Often applied in educational settings, public health, and healthcare delivery applications (Zhang et al., 2011)
Kellogg Evaluation Framework	Employs Logic Model including inputs, activities, outputs, outcomes, and impact to frame evaluation (W.K. Kellogg Foundation, 2017a)
CDC Program Evaluation Framework	Comprehensive evaluation model including stakeholder engagement, evaluation question development and prioritization, design, and evaluation with attention to sustainability and dissemination (CDC, 2024a)
Donabedian	Structure-process-outcome model used commonly to evaluate healthcare programs, quality metrics, and system performance (Berwick & Fox, 2016)
PRECEDE-PROCEED	Model used commonly in public health programs for planning, implementing, and evaluating programs (Green & Kreuter, 2005)
RE-AIM	Framework for the planning and evaluation according to 5 key outcomes: RE-AIM (2026)
CFIR	Framework to understand the context and factors affecting implementation within healthcare settings (Damschroder et al., 2022)
Kirkpatrick Model	4 level model commonly used to evaluate training, simulation, and workforce development in healthcare (Kirkpatrick Partners, 2024)

CDC, Centers for Disease Control and Prevention; CFIR, Consolidated Framework for Implementation Research; CIPP, Context, Input, Process, and Product; PRECEDE, Predisposing, Reinforcing, and Enabling Constructs in Educational/Environmental Diagnosis and Evaluation; PROCEED, Policy, Regulatory, and Organizational Constructs in Educational and Environmental Development; RE-AIM, Reach, Effectiveness, Adoption, Implementation, Maintenance.

Stakeholders and Collaborators

Engaging key collaborators and stakeholders is fundamental to conducting a rigorous and meaningful program evaluation within a DNP project. Leaders, clinicians, and staff who were involved in designing or implementing the program offer essential insight into its goals, operational processes, and underlying assumptions. Their perspectives help the DNP student understand how the program was

originally intended to function, how it is currently being carried out, and which contextual factors may influence its outcomes. Stakeholder input also enhances the relevance and credibility of the evaluation by ensuring that the evaluation questions, methods, and interpretations align with organizational priorities and the realities of the practice environment (Brown et al., 2023; Jeffs et al., 2022).

Because many programs are evaluated months or years after initial rollout, stakeholder collaboration is critical for reconstructing the program's history and understanding how it has evolved over time. This includes clarifying why the program was initiated, what outcomes leaders expected, and how the program was designed to operate. It also requires examining how real-world implementation may have diverged from the original plan due to workflow changes, staffing patterns, resource constraints, or shifting organizational priorities (Jeffs et al., 2022).

For example, when evaluating a falls-prevention program implemented on a cardiac step-down unit 6 months earlier, the DNP student must first understand the rationale for the program, the outcomes it aimed to achieve, and the protocols established at launch. The student must then compare these intentions with how the program is currently being operationalized. Engaging leaders, front-line nurses, and other stakeholders who participated in the program's design and implementation provides the historical context and operational insight needed to conduct an accurate, credible, and practice-relevant evaluation (Brown et al., 2023; Jeffs et al., 2022).

ENVIRONMENTAL OR CONTEXTUAL ASSESSMENT

Conducting an environmental or contextual assessment deepens the DNP student's understanding of the program by clarifying the conditions in which it operates. This analysis identifies key organizational factors such as culture, leadership engagement, resource availability, and external policy influences that shape how a program is implemented and how well it can perform. Research shows that interventions are more likely to fail when these contextual determinants are not recognized and addressed (McCreight et al., 2019; Nobrega et al., 2023).

An environmental assessment also helps the student understand the organization's structure, decision-making processes, and the roles of individuals involved in the program. These insights are essential for designing a comprehensive and realistic evaluation. Reflecting this importance, the Centers for Disease Control and Prevention (CDC) (2024a) begins with assessing the environmental context, underscoring that a clear understanding of the setting is foundational to any effective evaluation.

Table 2.5 summarizes the four contextual elements that include readiness for evaluation, evaluation capacity, stakeholders, and place which can help evaluators

TABLE 2.5 Evaluability and Environmental Context Factors in Program Evaluation

Context Factor	Description
Readiness for evaluation	Assess whether conditions are appropriate for conducting an evaluation. Determine if the program or clinical situation is sufficiently developed, if data systems are in place, and if stakeholders agree on the purpose and scope of the evaluation. Readiness (evaluability assessment) ensures the evaluation is feasible and meaningful. Stakeholders are interested in using the evaluation findings.
Evaluation capacity	Consider the organization and individuals' ability to support the evaluation. Identify staff skills, available time, data infrastructure, leadership support, and resources. Strong evaluation capacity increases the likelihood of producing credible, useful findings (CDC, 2024a).
Stakeholders	Identify individuals or groups who are invested in, influence, or may be affected by the evaluation. Determine if interest holders want to engage. Identify who can support the work, and who may be impacted by the findings. Early engagement strengthens collaboration and alignment.
Place	Refers to the setting or context in which the program and evaluation occur. Assess organizational culture, patient population, workflow patterns, resources, and environmental constraints. Recognizing the unique characteristics of the place ensures the evaluation is contextually appropriate and accurately interpreted.

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AQ: Please specify whether the reference citation "CDC (2024)" here refers to "CDC (2024a)" or "CDC (2024b)" or both.

Adapted from W.K. Kellogg Foundation. (2004). Logic model development guide: Using logic models to bring together planning, evaluation, and action (3rd ed.). W.K. Kellogg Foundation. https://www.naccho.org/uploads/downloadable-resources/Programs/Public-Health-Infrastructure/KelloggLogicModelGuide_161122_162808.pdf, Kellogg, 2017; CDC, 2024; Howard-Ruben, J., Hinch, B., & Livesay, S. (2025). The value of comprehensive evaluation DNP projects. *Journal of Professional Nursing*, 57, 133–138. <https://doi.org/10.1016/j.profnurs.2025.01.007>

understand the conditions in which a program operates and give insights to determine how to design an effective evaluation (CDC, 2024b).

Contextual Assessment Questions

Contextual evaluation questions focus on assessing the environment in which a program or intervention is implemented.

For example, when implementing a falls reduction program, evaluators must examine site-specific conditions such as staffing levels, workflow patterns, physical layout, equipment availability, and competing organizational priorities. These contextual factors may either support implementation or create barriers, including limited staff time, insufficient training resources, or environmental constraints that hinder consistent program delivery (Martiniano & Harasta, 2017; W.K. Kellogg Foundation, 2004, 2017a). Importantly, contextual evaluation questions are applicable not only to program evaluations but also to QI and EBP projects. Across all project types, identifying contextual and environmental influences is essential for designing evaluations that are sensitive to local conditions and capable of accurately capturing system-specific factors that affect program success (CDC, 2024b). Example of questions are:

- What is the organization's mission, vision, and strategic priorities?
- How does the project align with these priorities?
- Who were/are the key decision-makers and champions for this project?
- What is the level of leadership support for the evaluation?
- What are perceived barriers and facilitators to implementing a new initiative?
- What technology or data systems are currently in place for monitoring outcomes?
- Are there existing quality metrics that align with the project goals?

Problem and Purpose Statements in a Program Evaluation

Describing the underlying problem that warrants an evaluation and describing the purpose for the evaluation is foundational to any program evaluation. The problem statement explains *why* an evaluation is needed by describing either what is happening in the program that raises concern or a discrepancy or gap between what is observed and what is happening that warrants further investigation (Cole et al., 1995). The purpose statement specifies the *intent* of the evaluation by describing what the evaluation aims to determine. For example, the purpose of a program evaluation could be to: assess the effectiveness of the program in achieving its desired outcomes; identify barriers to achieving desired program outcomes; examine implementation fidelity, or inform scalability or expansion of the program to units or populations. The purpose statement then provides direction for selecting appropriate evaluation methods (Adams & Neville, 2020; CDC, 2024b). A theory of change or logic model can support this process by making explicit the assumptions that link program activities to expected outcomes and by translating the problem and purpose into a structured evaluation plan.

The clarity and precision of a problem statement and purpose statement directly influence the quality of a

healthcare program evaluation. Well-constructed problem statements help define the focus of the evaluation, guide the selection of appropriate methods, and ensure alignment with stakeholder priorities. In contrast, poorly written problem statements are vague, often solution-oriented, or judgmental, and fail to provide the specificity needed to guide meaningful evaluation or improvement efforts. Poorly written purpose statements often emphasize solutions, assign blame, or lack specificity regarding what the purpose of the initiative is. Clear descriptions of the problem and purpose are foundational to planning the evaluation (Adams & Neville, 2020; CDC, 2024b; Jeffs et al., 2022). **Table 2.6** describes examples of problem and purpose statements.

Synthesis of Relevant Literature

A thorough exploration of scholarly literature is a foundational step in initiating a DNP project. This is usually described as the background or review of literature content section of the DNP project. Before entering the development or design phase, students must build a comprehensive understanding of the existing evidence related to their project focus. Conducting a systematic review of the literature enables the student to clarify the scope of the problem, identify contributing factors to the project’s issue, and identify evidence-based strategies that may inform potential solutions (AACN, 2021; Hickey & Giardino, 2023; Melnyk & Fineout-Overholt, 2023).

The literature review identifies research and EBP or best practice recommendations, national standards, QI initiatives, and organizational benchmarks that support the project’s focus (Minnick et al., 2019; Turkson-Ocran et al., 2020). A scholarly foundation to the development of the

project enables the student to compare findings from the evaluation project to identify gaps between current practice and evidence-based standards. Such comparisons are critical for formulating recommendations that align with organizational priorities and improve patient outcomes (Hinch et al., 2020; Murphy et al., 2018). Furthermore, a robust literature review supports the development of evaluation questions, selection of an appropriate project framework, and identification of valid measures for assessing program effectiveness (CDC, 2024a; W.K. Kellogg Foundation, 2017b). Ultimately, integrating current evidence throughout the project lifecycle from planning to dissemination ensures that evaluation findings are credible, actionable, and aligned with the AACN Essentials for advanced nursing practice (AACN, 2021).

Evaluation Questions

Evaluation questions define what the evaluation is intended to learn and serve as the anchor for determining the purpose, scope, and direction of the assessment. They clarify the specific aspects of a program or system that need to be understood before any data are collected, ensuring that the evaluation remains focused on what matters most. Developing strong evaluation questions requires active engagement of collaborators and stakeholders, who help identify the issues they want the evaluation to address and keep attention centered on those priorities throughout the process (CDC, 2024a). All data collected during the evaluation are intentionally selected to answer these questions, allowing evaluators to generate clear, evidence-based findings that directly inform decision-making.

A key step in developing evaluation questions is to generate a broad list of possibilities and then narrow them

TABLE 2.6 Examples of Problem and Purpose Statements	
Well-Statesd Problem Statement	Weak Problem Statement
Six months after implementing the falls-prevention program, fall rates on the cardiac step-down unit have increased by 12%, suggesting possible gaps in implementation or workflow barriers.	The falls prevention program is not working and needs to be improved.
The diabetes education program has not achieved its A1C reduction target, and only 40% of patients complete all sessions, indicating potential barriers to participation.	Patients are not motivated to attend diabetes education classes, which is why outcomes are poor.
ED throughput times remain above the 180-minute benchmark despite the new triage protocol, raising concerns about inconsistent implementation.	The ED is too slow and staff need to follow the new triage protocol better.
Well-Statesd Purpose Statement	Poorly Statesd Purpose Statement
To assess implementation of the falls-prevention program and identify barriers contributing to persistent fall rates.	To fix the falls prevention program so fall rates go down.
To evaluate the diabetes education program’s impact on A1C levels and identify factors affecting patient participation.	To see if the diabetes program is good and make patients attend more sessions.
To examine fidelity to the triage protocol and determine whether variations contribute to prolonged ED throughput times.	To determine why the ED is slow and ensure staff follow the triage protocol correctly.

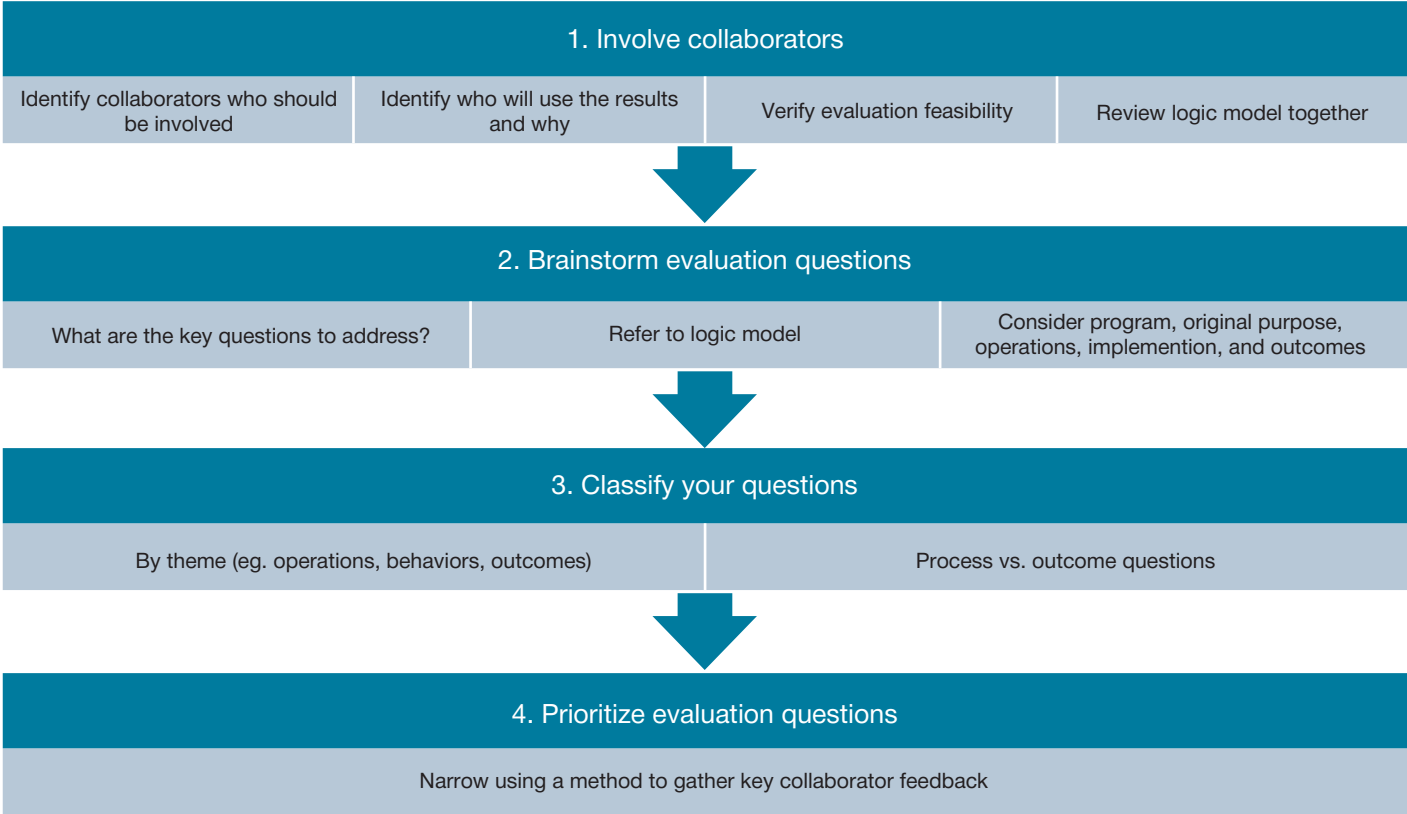


Figure 2.1 Steps to develop meaningful evaluation questions.
Source: Adapted from Centers for Disease Control and Prevention. (2024b, May). *Evaluation questions*. [Evaluation Questions](https://www.cdc.gov/library/research-guides/evaluation-questions.html). <https://www.cdc.gov/library/research-guides/evaluation-questions.html>

to those that are most relevant and feasible. Collaborators should be encouraged to think expansively about all aspects of the program that may warrant examination. As the list is refined, distinguishing process questions from outcome questions becomes essential. Process questions explore how the program is being implemented and should align with the inputs, activities, and outputs depicted in the logic model. Outcome questions focus on the program's effects and may address short-term, intermediate, or long-term outcomes. Prioritizing evaluation questions requires thoughtful consideration of several factors: the importance of the program to staff and stakeholders, the alignment of questions with program goals and objectives, whether the information generated will be useful and to whom, and whether the questions can be answered with available resources.

Figure 2.1 outlines the CDC's structured process for developing evaluation questions, guiding evaluators from initial collaboration through prioritization to ensure the questions are meaningful, feasible, and aligned with the program's logic model.

DATA COLLECTION IN PROGRAM EVALUATION

Data collection in program evaluation requires careful planning, attention to detail, and early decisions about the types of information needed to answer evaluation

questions (W.K. Kellogg Foundation, 2004, 2017a). Before collecting any data, the evaluation team must clearly define what information is necessary for each question. For instance, if an evaluation question asks, "To what extent is the program being implemented as intended?" the evaluators determine the appropriate measures, indicators, and data collection tools—such as surveys, observations, or checklists—needed to accurately assess implementation fidelity. Thoughtful selection of data collection approaches ensures that the evaluation generates reliable, valid, and actionable information.

Table 2.7 shows evaluation questions and data collection methods needed to answer the evaluation questions.

DATA COLLECTION TOOLS IN PROGRAM EVALUATION

Data collection tools are the instruments used to gather information based on the evaluation methods selected. Common examples include surveys, interview guides, observation checklists, and data extraction forms. The evaluation team first determines what data are needed to answer the evaluation questions and then selects appropriate tools to collect that data. For instance, if the goal is to understand staff perceptions of a program's effectiveness, suitable tools could include structured surveys, one-on-one

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TABLE 2.7 Evaluation Questions and Corresponding Data Collection Methods

Evaluation Question	Data Collection Methods Needed
<i>To what extent is the program being implemented as intended?</i>	Direct observation of work-flows; staff interviews; implementation checklists; document review (protocols, training materials)
<i>What outcomes have changed since the program was introduced?</i>	EMR data (clinical outcomes, utilization metrics); quality dashboards; pre-/post-intervention data comparison
<i>What barriers or facilitators influence staff participation in the program?</i>	Semi-structured staff interviews; focus groups; brief surveys; field notes from observations

EMR, electronic medical record.

interviews, or focus groups. By linking the type of data to the appropriate tool, evaluators ensure the information collected is relevant, reliable, and actionable.

Quantitative Data

Quantitative data are numerical measurements used to assess program performance, outcomes, and trends over time. Examples include clinical indicators, patient outcomes, service utilization rates, and other metrics that can be counted or measured. These data allow evaluators to track change, compare results against benchmarks, and determine whether a program is achieving its intended effects. Collecting quantitative data requires structured and standardized tools, such as surveys with closed-ended questions, audits, or electronic health record (EHR) extraction forms, to ensure consistent measurement across participants and settings. Well-designed quantitative tools enhance reliability and validity by minimizing measurement error and bias, thereby strengthening the credibility and generalizability of evaluation findings (Hickey & Giardino, 2023; W.K. Kellogg Foundation, 2004, 2017a).

Quantitative Data Collection Tools

- **Electronic medical record (EMR) data extraction forms**
Standardized templates used to pull clinical indicators, utilization metrics, and outcome data consistently across patients and time periods.
- **Validated surveys with closed-ended items**
Instruments using Likert scales, multiple-choice questions, or numerical ratings to measure knowledge, attitudes, behaviors, or satisfaction.

- **Structured checklists**

Tools that capture the presence or absence of specific behaviors, processes, or adherence to protocols.

- **Audit tools**

Instruments used to review documentation, compliance, or performance indicators (e.g., chart audits, medication reconciliation audits).

- **Administrative or quality-dashboard data forms**

Templates for extracting unit-level or organizational performance metrics (e.g., throughput times, fall rates, infection rates).

- **Standardized measurement instruments**

Tools such as validated clinical scales (e.g., pain scales, functional status measures) used to quantify patient outcomes.

- **Pre-/post-test instruments**

Tools used to measure change in knowledge, skills, or performance before and after an intervention.

Qualitative Data

Qualitative data complement numerical findings by capturing the experiences, perceptions, and contextual factors that shape how a healthcare program functions. These data help evaluators understand why a program is or is not achieving its intended outcomes and illuminate barriers, facilitators, and unintended consequences of the program. Qualitative data collection tools must be designed to support systematic, ethical, and structured collection processes that align with the evaluation questions. The tools enable evaluators to document behaviors, interactions, and workflow patterns (Hickey, 2022; W.K. Kellogg Foundation, 2017a) so that the data provides an understanding of program performance and the contextual influences that shape program outcomes.

After data collection and prior to beginning analysis, evaluators conduct a pre-analysis preparation phase in which the data is reviewed and organized to ensure completeness, accuracy, and consistency. This process may involve checking for missing information, clarifying ambiguous responses, and formatting or categorizing data as needed, so that it is well-structured and ready for thorough qualitative analysis (Manikandan, 2010).

Qualitative Data Collection Tools

- **Semi-structured interview guides**

Frameworks with open-ended prompts that allow exploration of stakeholder experiences, perceptions, and contextual factors.

- **Focus group protocols**

Structured guides used to facilitate group discussions and capture collective insights about program functioning or barriers.

- **Open-ended survey items**

Written prompts that allow respondents to describe experiences or perspectives in their own words.

- **Observation templates or field-note forms**
Tools used to document behaviors, interactions, workflow patterns, and environmental factors during direct observation.
- **Document review templates**
Structured forms for analyzing policies, protocols, training materials, or meeting minutes to understand program design and implementation.
- **Reflective journals or logs**
Tools used by participants or evaluators to capture ongoing experiences, challenges, or observations over time.
- **Storytelling or narrative-capture tools**
Methods for gathering patient or staff narratives to illuminate lived experiences and contextual influences.

Analysis of Data

Decisions about data analysis are largely based on the questions to be answered, the design, and the level of data collected (nominal, ordinal, interval, and ratio). Consultation with a statistician at the planning stage may help when available to ensure the data collected and the analysis plan are appropriate for the questions that you want the evaluation to answer. Program evaluation, similar to any type or project, commonly uses descriptive, bivariable, or multivariable analysis or a combination of these, depending on the scope of the evaluation (Jones et al., 2021). Descriptive analysis produces measures of means, medians, frequencies, and percentages that are sufficient for many evaluations. Bivariable analysis constructs associations (or differences) between two items; for example, a hand-washing program and nosocomial infections. Multivariable analysis employs more complex statistical techniques, such as multiple regressions, to determine the effect of a program, the direction of the effect, and how variables act separately and together. Once data analysis is completed, the findings are interpreted in light of the evaluation questions (W.K. Kellogg Foundation, 2017a). For DNP and smaller projects, the analysis of evaluation data tend to be simpler, most often using descriptive and nonparametric statistics. The interpretation of the findings in light of the expected outcomes should be carefully considered by the evaluator and the team. (See Chapter 7, “Data Analysis and Display Improvement Initiatives,” for in-depth discussion of data analysis and data display.)

RECOMMENDATIONS FROM EVALUATION FINDINGS

After analyzing evaluation data, evaluators identify gaps, barriers, and areas for improvement and develop recommendations to address them. Involving key stakeholders in data interpretation and recommendation development ensures that findings reflect the organizational context

and that proposed actions are feasible, relevant, and sustainable. Recommendations should directly align with the evaluation’s purpose and questions, be specific and actionable, and reflect the strength and certainty of the evidence. Additionally, they should consider stakeholder values, organizational priorities, and potential resource constraints. Prioritizing recommendations can help guide decision-making and support effective implementation (CDC, 2024a, 2024b).

DISSEMINATE FINDINGS TO INTEREST GROUPS/ KEY COLLABORATORS

Dissemination of findings and recommendations to key interest holders and to the broader academic community is an important piece to complete an evaluation. Dissemination may include written reports or oral presentations and often includes both. The information and delivery style is tailored to the needs of the audience and should be delivered in a manner that allows for dialogue. Once the internal dissemination of findings is complete, consider if the findings from the evaluation can inform a broader discussion about care delivery. Evaluation findings may lend itself to a manuscript submitted to a professional journal, or other formats such as a podcast or “op-ed” writing online or in print. Often, additional work is required to frame the evaluation and findings within the context of what is already published and may include lessons learned by the evaluator.

CONCLUSION

Program evaluation is a key skill for DNP-prepared nurses and may be employed as the main purpose of a DNP project while obtaining a DNP degree. When conducted, program evaluation should be both rigorous and systematic, employing the use of an evaluation framework or methodology and answer key evaluation questions. While many of the steps to complete a DNP project with an evaluation focus are similar to an implementation-focused project, some key differences exist.

DISCUSSION QUESTIONS

1. **Scenario:** A community clinic has implemented a new diabetes self-management program. Early reports indicate that only 50% of participants attend all sessions. The clinic wants to understand why and whether the program is achieving its intended outcomes. **Question:** As a DNP student tasked with evaluating the program, which type(s) of evaluation would you select, and what methodology would best address both participation barriers and program effectiveness? Explain your reasoning.

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2. **Scenario:** An urban hospital plans to implement a falls prevention program on multiple units. Initial data suggest variable fall rates across units. **Question:** What contextual factors should you assess before analyzing program data, and how might these factors influence your evaluation approach and recommendations?
3. **Scenario:** Following evaluation of a hypertension screening program, data show inconsistent follow-up for patients with elevated blood pressure, despite high initial screening rates. **Question:** How would you analyze the data and develop actionable recommendations? What considerations ensure your recommendations are feasible and evidence-informed?
4. How is program evaluation defined, and what is its primary purpose in healthcare?
5. Why is it important to select the appropriate type of evaluation (e.g., formative, summative, outcome) for a program, and how does the program stage influence this choice?

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REFERENCES

- American Association of Colleges of Nursing. (2021). *The essentials: Core competencies for professional nursing education*. Author. <https://www.aacnnursing.org/Essentials>
- Adams, J., & Neville, S. (2020). Program evaluation for health professionals: What it is, what it isn't and how to do it. *International Journal of Qualitative Methods*, 19, 1–11. <https://doi.org/10.1177/1609406920964345>
- Berwick, D., & Fox, D. M. (2016). Evaluating the quality of medical care: Donabedian's classic article 50 years later. *The Milbank Quarterly*, 94(2), 237–241. <https://doi.org/10.1111/1468-0009.12189>
- Brown, D., Carter, K., & Patel, R. (2023). Strengthening practice-based inquiry through stakeholder engagement in nursing quality improvement initiatives. *Journal of Nursing Care Quality*, 38(2), 123–130.
- Centers for Disease Control and Prevention. (2024a). CDC program evaluation framework, 2024. *MMWR. Recommendations and Reports*, 73(6), 1–37. <https://doi.org/10.15585/mmwr.r7306a1>
- Centers for Disease Control and Prevention. (2024b, May). *Evaluation questions*. <https://www.cdc.gov/library/research-guides/evaluation-questions.html>
- Cole, G. E., Pogostin, C. L., Westover, B. J., Rios, N. M., & Collier, C. B. (1995). Addressing problems in evaluating health-relevant programs through systematic planning and evaluation. *Risk: Health, Safety & Environment*, 6(1), 37–51. https://www.orau.gov/hsc/cdcynergy30/ba/Content/activeinformation/resources/Addressing_Problems.pdf
- Damschroder, L. J., Reardon, C. M., Widerquist, M. A. O., & Lowery, J. (2022). The updated Consolidated Framework for Implementation Research based on user feedback. *Implementation Science*, 17(1), 75. <https://doi.org/10.1186/s13012-022-01245-0>

- Fain, J. A. (2020). *Reading, understanding, and applying nursing research* (6th ed.). F.A. Davis.
- Fitzpatrick, J. L., Sanders, J. R., Worthen, B. R., & Wingate, L. A. (2023). *Program evaluation: Alternative approaches and practical guidelines* (5th ed.). Pearson.
- Frye, A. W., & Hemmer, P. A. (2012). Program evaluation models and related theories: AMEE Guide No. 67. *Medical Teacher*, 34(5), e288–e299. <https://doi.org/10.3109/0142159X.2012.668637>
- Green, L. W., & Kreuter, M. W. (2005). *Health program planning: An educational and ecological approach* (4th ed.). McGraw-Hill.
- Hickey, J. V. (2022). Planning a program or project evaluation. In J. V. Hickey, E. R. Giardino (Eds.), *Evaluation of health care quality for DNP* (3rd ed.). Springer Publishing.
- Hickey, J. V., & Giardino, E. R. (2023). The ongoing conundrum about DNP project variability. *Journal of Professional Nursing*, 49, 70–78. <https://doi.org/10.1016/j.profnurs.2023.08.012>
- Hinch, B. K., Livesay, S., Stifter, J., & Brown, F. (2020). Academic-practice partnerships: Building a sustainable model for DNP projects. *Journal of Professional Nursing*, 36(6), 569–578. <https://doi.org/10.1016/j.profnurs.2020.08.008>
- Howard-Ruben, J., Hinch, B., & Livesay, S. (2025). The value of comprehensive evaluation DNP projects. *Journal of Professional Nursing*, 57, 133–138. <https://doi.org/10.1016/j.profnurs.2025.01.007>
- Idzik, S., Buckley, K., Bindon, S., et al. (2021). Lessons learned using logic models to design and guide DNP projects. *Nurse Educator*, 46(5), E127–E131. <https://doi.org/10.1097/NNE.0000000000001025>
- James Bell Associates. (2018). *Formative evaluation toolkit: A step-by-step guide and resources for evaluating program implementation and early outcomes*. Children's Bureau, Administration for Children and Families, U.S. Department of Health and Human Services. <https://www.jbassoc.com/wp-content/uploads/2019/01/Formative-Evaluation-Toolkit.pdf>
- Jeffs, L., Beswick, S., & Lo, J. (2022). Partnering with stakeholders to enhance evaluation capacity and improve healthcare programs. *Journal of Clinical Nursing*, 31(19–20), 2801–2812.
- Jones, M., deValpine, M., McDonald, M., & Schubert, C. (2021). Use of statistical tests in Doctor of Nursing Practice projects. *The Journal for Nurse Practitioners*, 17, 1118–1121. <https://doi.org/10.1016/j.nurpra.2021.06.006>
- Kirkpatrick Partners. (2024). *The Kirkpatrick model*. Kirkpatrick Partners. <https://www.kirkpatrickpartners.com/the-kirkpatrick-model/>
- Manikandan S. (2010). Preparing to analyse data. *Journal of Pharmacology & Pharmacotherapeutics*, 1(1), 64–65. <https://doi.org/10.4103/0976-500X.64540>
- Martiniano, R., & Harasta, E. (2017). *Evaluation guide for HRSA project officers*. Health Workforce Technical Assistance Center, Center for Health Workforce Studies, School of Public Health, University at Albany, SUNY. https://www.healthworkforceta.org/wp-content/uploads/2019/09/Evaluation_Guide_for_HRSA_Project_Officers.pdf
- McCreight, M. S., Rabin, B. A., Glasgow, R. E., Ayele, R. A., Leonard, C. A., Gilmartin, H. M., Frank, J. W., Hess, P. L., Burke, R. E., & Battaglia, C. T. (2019). Using the Practical, Robust Implementation and Sustainability Model (PRISM) to qualitatively assess multilevel contextual factors to help plan, implement, evaluate, and disseminate health services programs. *Translational Behavioral Medicine*, 9(6), 1002–1011. <https://doi.org/10.1093/tbm/ibz085>
- Melnik, B. M., & Fineout-Overholt, E. (2023). *Evidence-based practice in nursing & healthcare: A guide to best practice* (5th ed.). Wolters Kluwer.
- Minnick, A. F., Kleinpell, R., & Allison, T. L. (2019). Reports of three organizations' members about DNP project experiences and outcomes. *Nursing Outlook*, 67(6), 671–679. <https://doi.org/10.1016/j.outlook.2019.05.012>
- Murphy, M. P., Staffileno, B. A., Hinch, B., & Carlson, E. (2018). Promoting clinical scholarship in DNP programs. *The Journal for Nurse Practitioners*, 14(2), e31–e39. <https://doi.org/10.1016/j.nurpra.2017.12.003>
- Nobrega, S., Edwards, K., Ghaziri, M. E., Giacobbe, L., Rice, S., & Punnett, L. (2023). A protocol to assess contextual factors during program impact evaluation: A case study of a STEM gender equity intervention

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- in higher education. *American Journal of Evaluation*, 45(4), 592–609. <https://doi.org/10.1177/10982140231152281>
- Ogunyemi, K. O., McNabb, S., Lokossou, V., Sogbossi, L. S., Nyenswah, T., & Ohuabunwo, C. (2025). Developing a new pragmatic tool for assessing contextual fit and feasibility of evidence-based interventions towards effective implementation in global health. *BMJ Global Health*, 10(4), e015931. <https://doi.org/10.1136/bmjgh-2024-015931>
- Partelow, S. (2023). What is a framework? Understanding their purpose, value, development and use. *Journal of Environmental Studies and Sciences*, 13(3), 510–519. <https://doi.org/10.1007/s13412-023-00833-w>
- RE-AIM. (2026). *What is RE-AIM?* <https://re-aim.org/learn/what-is-re-aim/>
- Rossi, P. H., Lipsey, M. W., & Henry, G. T. (2019). *Evaluation: A systematic approach* (8th ed.). Sage.
- Stufflebeam, D. L., Foley, W. J., Gephart, W. J., Guba, E. G., Hammond, R. L., Merriman, H. O., & Provus, M. M. (1971). *Educational evaluation and decision making*. Peacock.
- Suskie, L. (2018). *Assessing student learning: A common sense guide* (3rd ed.). Jossey-Bass.
- Turkson-Ocran, R.-A. N., Spaulding, E. M., Renda, S., Pandian, V., Rittler, H., Davidson, P. M., Nolan, M. T., & D'Aoust, R. (2020). A 10-year evaluation of projects in a doctor of nursing practice programme. *Journal of Clinical Nursing*, 29(21–22), 4090–4103. <https://doi.org/10.1111/jocn.15435>
- U.S. Environmental Protection Agency. (2025). *Program evaluation at EPA*. <https://www.epa.gov/evaluate/program-evaluation-epa>
- W.K. Kellogg Foundation. (2004). *Logic model development guide: Using logic models to bring together planning, evaluation, and action* (3rd ed.). W.K. Kellogg Foundation. https://www.naccho.org/uploads/downloadable-resources/Programs/Public-Health-Infrastructure/KelloggLogic-ModelGuide_161122_162808.pdf
- W.K. Kellogg Foundation. (2017a). *The step-by-step guide to evaluation: How to become savvy evaluation consumers*. https://www.betterevaluation.org/sites/default/files/WKKF_StepByStepGuideToEvaluation_smaller.pdf
- W.K. Kellogg Foundation. (2017b). *W.K. Kellogg Foundation logic model development guide*. <https://wkkf.issuelab.org/resource/logic-model-development-guide.html>
- Weiss, C. H. (1998). *Evaluations: Methods for studying programs and policies* (2nd ed.). Prentice Hall.
- White, K. M., Dudley-Brown, S., & Terhaar, M. F. (2021). *Translation of evidence into nursing and health care* (4th ed.). Springer Publishing.
- Wholey, J. S., Hatry, H. P., & Newcomer, K. E. (2015). *Handbook of practical program evaluation* (4th ed.). Jossey-Bass.
- Zhang, G., Zeller, N., Griffith, R., Metcalf, D., Williams, J., Shea, C., & Misulis, K. (2011). Using the Context, Input, Process, and Product Evaluation Model (CIPP) as a comprehensive framework to guide the planning, implementation, and assessment of service-learning programs introduction and review of literature. *Journal of Higher Education Outreach and Engagement*, 15(4), 57. <https://files.eric.ed.gov/fulltext/EJ957107.pdf>

DIGITAL-ONLY CONTENT

ABSTRACT

Ethical, legal, and regulatory considerations are integral to the design, implementation, and dissemination of evaluation and quality improvement initiatives. This chapter examines ethical principles including beneficence, justice, respect for persons, and transparency, and applies them to healthcare evaluation activities. Distinctions between quality improvement, evaluation, and research are clarified in relation to regulatory oversight, institutional review board (IRB) requirements, and data governance. The chapter also addresses issues such as patient privacy, informed consent, data security, and responsible dissemination of findings. By understanding these considerations, DNP-prepared nurses are equipped to conduct evaluations that protect patients, uphold professional integrity, and comply with organizational and regulatory standards while advancing meaningful system improvement.

KEYWORDS

ethics, quality improvement, program evaluation, regulatory requirements, IRB, data privacy

MESH TERMS

Ethics, Program Evaluation, Quality Improvement, Informed Consent, Confidentiality, Health Policy, Nursing Practice, Nursing, Graduate